

Standard Operating Procedure

Early Warning Signs and Escalation

«Hospital Name» — Supports AAC.5.e

SOP No.	[Hospital to define]	Version	1.0
Effective Date	_____	Review Frequency	Annual, or on change
Prepared By	Quality Coordinator Name: _____ Signature: _____		
Approved By	Medical Superintendent Name: _____ Signature: _____		

1. Purpose

This SOP describes how «Hospital Name» identifies early warning signs of clinical deterioration and ensures prompt escalation and intervention, every time, without exception.

2. Scope

Applies to every doctor, nurse, and direct clinical caregiver monitoring in-patients, day-care patients, and emergency patients.

3. Responsibility

Any Staff Member Observing a Trigger: escalates immediately — this is not limited to nursing or doctors on duty.

On-Call / Responding Doctor: acts on the escalation promptly and documents the response.

Quality Coordinator: reviews early-warning system effectiveness at a defined interval and investigates every failure-to-escalate event.

4. Procedure

1. Use the hospital's defined physiological parameters — which may include vital parameters, airway, circulation, neurological status, and any other concern raised by staff or the patient/family — to watch for early warning signs, tailored by speciality or age where the hospital has defined such tailoring. A recognised, structured basis for this kind of scoring is the National Early Warning Score 2 (NEWS2) — see Section 7, Reference 1 — which the hospital may adopt in full or in part.
2. The moment a trigger is met, inform appropriate medical personnel immediately — do not defer this for non-urgent work of any kind.
3. Start prompt, appropriate intervention while escalation is happening, not after.
4. Document the trigger, the time informed, who responded, and what action was taken, in the Escalation Record.
5. The Quality Coordinator reviews early-warning system effectiveness at the interval they define and document, and investigates every failure-to-escalate event as an incident with root-cause analysis.

5. Stop-Work Authority

Do not leave a deteriorating patient without escalation when an early-warning sign or defined physiological trigger indicates change or deterioration.

If a trigger is met and appropriate medical personnel have not yet been informed and acted, escalate immediately per this SOP and start prompt intervention without delay.

Stop-work applies to the failure to escalate — it never blocks emergency life-saving measures, which continue while escalation happens.

The person responsible tells the treating doctor, and if there is no timely response, the Medical Superintendent, the same shift. Refusing to leave a deteriorating patient unescalated is not a disciplinary matter.

6. Records

Supported by Records 11–14 of the AAC.5 Required Records pack: the Early Warning Guideline Reference, Escalation Record, Early-Warning Effectiveness Review Notes, and Failure-to-Escalate Incident/RCA Record.

7. Related Documents

- HCO_AAC_5_v2_REWRITE_DRAFT — AAC.5 policy
- SOP — Patient Re-assessment and Care Plan Modification

8. Review

Reviewed annually, and immediately after any failure-to-escalate event.

9. References

- Royal College of Physicians. (2017). National Early Warning Score (NEWS) 2: Standardising the assessment of acute-illness severity in the NHS. Updated report of a working party.
<https://www.rcplondon.ac.uk/projects/outputs/national-early-warning-score-news-2>